PreOpsis · Seller sheet

INTERNAL · DO NOT LEAVE BEHIND

These meetings are a **sense check**. The goal is to learn how the hospital's pre-operative screening (POS) works today, what hurts, and whether PreOpsis would fix it. Listen more than you pitch. The brochure describes the full offer; this sheet tells you what exists today, so you never promise a date or a certificate in the room.

Discovery questions

1. How does your POS work today: a questionnaire in the HiX or Epic portal, Dräger E-POS, paper, or by phone?
2. What share of patients is screened without a visit to the POS clinic? What would you like it to be?
3. Roughly how many POS screenings do you do a year, and at how many locations?
4. Who validates the answers: POS nurse, anaesthetist, physician assistant?
5. How often is an operation moved or cancelled because the screening wasn't complete in time?
6. Which EHR and version do you run? Who owns integrations (applicatiebeheer, IT)? Do you use HiX's triage?
7. Who changes the questionnaire today, and how long does a change take?
8. How do patients sign in today (DigiD through the portal)? Would your FG or CISO accept a personal link plus date of birth?
9. Which languages do your patients need? How do you handle low literacy (B1 texts)?
10. Do you collect PROMs for the LROI? With which tool, and who chases non-responders?
11. Who would decide and who holds the budget: anaesthesiology, OK management, IT, the board? Would this go to tender?

Objections, answered honestly

"HiX already does this."
Many Dutch hospitals use HiX questionnaires and its triage (Diakonessenhuis computes a provisional ASA in HiX). Ask what is hard: changing content, languages, the phone experience, PROMs. PreOpsis gives the POS team its own editor with test cases and sends the result back into the EHR.

"We use E-POS."
Dräger E-POS is used at Noordwest and WZA. Ask what they would change: flexibility of content, languages, recovery follow-up in the same system.

"Is this a medical device?"
Suggested ASA and automatic triage are decision support. Under EU MDR Rule 11 that is likely Class IIa, which needs a notified body. PreOpsis has no CE mark yet. Say: a clinician validates every outcome, and we will complete MDR conformity before triage goes live in the Netherlands.

"NEN 7510? ISO 27001?"
Not certified. NEN 7510 is mandatory for the hospital's own processing. Explain what is in place (data separated per hospital, audit log of every view, MFA for staff, encryption) and ask for their supplier requirements so we can plan for them.

"You're a start-up."
Offer a contained start with one specialty, a named contact, and full export of their data and questionnaires. The hospital owns its content.

"Patients use DigiD here."
The link plus date of birth is built for speed: tap the SMS and answer the first question within seconds. Note DigiD as a requirement if they insist; it is not built.

Never say

- ✕ "We are NEN 7510 / ISO 27001 certified"
- ✕ "Hospital X uses it"
- ✕ Any percentage or time saving as our result
- ✕ "It has a CE mark" or "it's not a medical device"
- ✕ "It's integrated with HiX / Epic today"
- ✕ A delivery date for anything not Live

What exists today

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Live means built and tested on a test environment with made-up data. No hospital uses PreOpsis yet, and it holds no real patient data. **Planned** means it is in the build plan but not started. **Not in plan** means nobody has scoped it. Never give a delivery date for anything that isn't Live.

BROCHURE PROMISE	STATUS	WHAT TO SAY
Questionnaire editor: logic, scores, codes, test cases, versions	Live	Show it. This is the strongest demo.
Phone questionnaire, link + date of birth, autosave, one question per screen	Live	Show it on a phone.
SMS and email invites, reminders on day 2 and day 5, "not responding"	Live	Sending SMS works to UK numbers only today; Dutch numbers need set-up.
Validation per section, validated vs patient answers, POS summary	Live	Printing works; the PDF document layout is Planned.
Several locations, roles, MFA for staff, audit log	Live	Audit log exports to CSV.
Worklists	Partly	"Ready for review" and "not responding" are live; "consultation needed" is Planned.
Suggested ASA, STOP-Bang, Apfel, automatic triage	Planned	Planned; MDR applies (see objections).
Earlier answers pre-filled	Planned	Planned.
Dutch, Turkish, Arabic, Polish for patients	Planned	Translation workflow exists in the editor; Dutch and a language switch for patients are not built.
Recovery check-ins and PROMs (EQ-5D-5L, Oxford, HOOS-PS/KOOS-PS)	Planned	Planned. EQ-5D needs a paid licence from EuroQol.
EHR integration with HiX, Epic, Nexus (FHIR, HL7)	Not in plan	"Set up per hospital." Ask what interfaces they already run. Never promise a date.
BSN as identifier	Not in plan	The identifier is designed to be per country; only the NHS number is built.
Hosting within the EU	Not set up	Runs in AWS London today. One deployment per country is the design; an EU region needs set-up.
GDPR (AVG) data processing agreement	Planned	We sign one as processor; the template isn't written yet.

Dutch market facts and sources

EHR market: ChipSoft (HiX) about 71%, Epic 14%, Nexus 11% of hospitals (M&I/Partners, 1 Jan 2024). dutchitchannel.nl

Digital POS in HiX: Diakonessenhuis screens 55% fully digitally since Feb 2024; ASA 1–2 assessed digitally, ASA 3–4 by phone or visit. conclusion.com

E-POS in use: WZA (from June 2024, plus the Thuismeten app for aftercare) and Noordwest Ziekenhuisgroep. skipr.nl · nwz.nl

No questionnaire, no surgery date: ZGT requires the questionnaire (DigiD login) before a POS appointment. zgt.nl

LROI PROMs: NRS pain, EQ-5D-5L, HOOS-PS/KOOS-PS; the NOV has proposed making Oxford scores mandatory. Hip measured at 3 and 12 months, knee at 6 and 12. orthopeden.org

EU MDR Rule 11: software informing diagnostic or therapeutic decisions is at least Class IIa. Check MDCG 2019-11. namsa.com

BSN: under the Wbsn-z, care providers must record and use the BSN when exchanging data. Verify the current text on wetten.overheid.nl.

NEN 7510/7512: mandatory for care providers (Besluit elektronische gegevensverwerking door zorgaanbieders); no rule on where data is hosted. From our internal research notes.

Languages: ZGT offers translations including English, Polish, Turkish, Arabic, Tigrinya and Ukrainian. zgt.nl